

Limye' Foundation Comprehensive Service Blueprint: Version 2.0 - Integrated Veteran Services Initiative

Executive Summary

Limye' Foundation is a non-profit organization positioned to transform the delivery of essential support services across Alabama. This updated blueprint details the integration of a new, high-impact focus on the state's veteran population, building upon our established framework of serving seniors, at-risk youth, and families in crisis. The core of this expansion is **"Operation Home Base,"** a synergistic initiative designed to address the most critical Social Determinants of Health (SDOH) for veterans through an innovative integration of stable housing, medically tailored nutrition, and essential transportation.

This initiative is anchored by strategic partnerships that create a sustainable and scalable service ecosystem. The **"20/59 Ventures Housing Initiative"** will be actualized through a formal partnership with a proven local developer, such as Duckworth-Morris Real Estate, to provide a dedicated inventory of safe, affordable housing units for veteran clients.¹ This stable housing creates a foundational client base for our other services. Our nutritional and transportation services will be financially sustained through a contractual relationship with the U.S. Department of Veterans Affairs (VA), primarily via its Community Care Network (CCN) administrator for Alabama, Optum.³ Furthermore, collaboration with established Veteran Service Organizations (VSOs) will enable us to access specialized funding like the VA's Highly Rural Transportation Grants (HRTG) to extend our reach into underserved communities.⁴

This veteran-focused initiative represents more than a mission expansion; it introduces a new, diversified revenue stream through VA reimbursements, significantly strengthening the foundation's long-term financial health and sustainability as projected in our original plan.⁶ By integrating these core services, Operation Home Base will create a powerful, replicable blueprint for veteran support that reduces

homelessness, improves health outcomes, and illuminates clear pathways to self-sufficiency for those who have served our nation.

Section 1: Organization Overview and Mission

Foundation Identity

- **Legal Name:** Limye' Foundation
- **Previous Name:** Porch Light
- **Tax Status:** 501(c)(3) Non-Profit Organization
- **Primary Location:** Tuscaloosa, Alabama
- **Service Area:** Statewide Alabama with focus on West Alabama region ⁶

Mission Statement (Revised)

Limye' Foundation empowers vulnerable populations, **including our nation's veterans**, through comprehensive "prep services" that bridge critical gaps in healthcare access, nutrition, **housing**, and community support, creating pathways to self-sufficiency and improved quality of life.

Vision Statement (Revised)

To be the trusted catalyst for transformative community support, connecting Alabama's most vulnerable citizens, **with a dedicated focus on veterans**, to life-changing healthcare, nutrition, and comprehensive wrap-around services.

Core Values

- **Human Dignity:** Every person deserves respect and quality care.
- **Cultural Humility:** Honoring diverse backgrounds and experiences.
- **Innovation:** Pioneering solutions to complex social challenges.
- **Collaboration:** Building bridges between services and communities.
- **Accountability:** Measuring and maximizing our impact.
- **Service & Sacrifice:** Honoring the unique contributions and needs of veterans and their families. ⁶

Core Service Categories

1. **Healthcare Navigation & Social Determinants of Health (SDOH):** Community Health Worker (CHW) programs, Medicare SDOH risk assessments, care coordination, health literacy, and chronic disease management support.
2. **Nutritional Services:** Medically Tailored Meals (MTM), senior meal programs, institutional food services, and nutrition education.
3. **Transportation Services:** Non-Emergency Medical Transportation (NEMT), wheelchair-accessible transport, general community transportation, and meal delivery logistics.
4. **Educational & Youth Services:** Youth diversion programs, after-school enrichment, life skills training, and career readiness.
5. **Housing & Community Support:** This category is elevated in prominence with the new veteran initiative, encompassing transitional housing assistance, rapid rehousing, homelessness prevention, family resource centers, and benefits enrollment assistance.
6. **Institutional Services:** Correctional facility support, group home services, and school system partnerships, with **Military and Veteran Support Services** now established as a primary service line. ⁶

Section 2: Comprehensive Market Analysis

This section retains the original, detailed analysis of procurement opportunities within

Alabama for inmate meal services, senior meal services, youth services, educational support, medical services, and community development.⁶ The following subsection is added to address the new strategic focus on veteran services.

H. Veteran Service Opportunities

Market Size and Demographics

Alabama is home to a significant veteran population, with major Department of Veterans Affairs (VA) medical centers located in Tuscaloosa and Birmingham, which serve as critical hubs for healthcare in the region.⁷ These facilities, along with their associated community-based outpatient clinics, represent the primary points of service delivery and, therefore, the primary market for Limye' Foundation's proposed veteran-focused initiatives.

The VA as a Primary Payer and the Community Care Network (CCN)

The VA MISSION Act of 2018 fundamentally transformed how the VA provides care, significantly expanding veterans' access to services from community-based providers. This positions the VA as a major healthcare and social services contractor for organizations like Limye' Foundation. A critical component of this system is the VA's use of Third-Party Administrators (TPAs) to manage its Community Care Network (CCN).³

Alabama falls within CCN Region 3, which is exclusively administered by Optum Public Sector Solutions, Inc..³ This means that for a veteran to be referred to a community provider for a covered service, and for that provider to be paid, the provider must be credentialed and contracted with Optum. Veterans become eligible for community care based on specific criteria, such as the service not being available at a VA facility, excessive drive times (e.g., >30 minutes for primary care, >60 minutes for specialty care), or extended wait times (e.g., >20 days for primary care, >28 for specialty

care).¹⁰ Once a veteran is deemed eligible, the VA issues a referral through the CCN to an approved in-network provider.

Consequently, for Limye' Foundation to become a billable provider for VA-referred Non-Emergency Medical Transportation (NEMT) and Medically Tailored Meals (MTM), it must successfully navigate Optum's specific credentialing and contracting process. The Foundation's primary partnership for these reimbursable services will be with Optum, making this relationship a cornerstone of the initiative's financial model and sustainability.

Specific Service Market: Non-Emergency Medical Transportation (NEMT)

The VA offers several transportation solutions, including the Beneficiary Travel program for mileage reimbursement and the Veterans Transportation Service (VTS), which uses VA-operated vans.¹¹ However, gaps in these services create a significant market opportunity. A review of the Tuscaloosa VA Medical Center's website reveals that its Disabled American Veterans (DAV) van service, a key transport option, is currently noted as "not available at this time".¹²

This documented service disruption presents a compelling and immediate entry point for Limye's "Vets-in-Motion" program. It provides concrete evidence of a service gap that the Foundation is uniquely positioned to fill. Limye' can approach the Tuscaloosa VA's Community Care Office (contact number: 205-554-2926) with a ready-made solution to this known problem, positioning itself as a responsive and capable partner for both scheduled NEMT and overflow capacity.¹³

Specific Service Market: Medically Tailored Meals (MTM)

The veteran population has a disproportionately high rate of chronic health conditions, with 72% suffering from at least one such condition.¹⁴ Recognizing this, the VA has launched a high-level strategic "Food is Medicine" initiative, supported by VHA Directives and partnerships with influential organizations like The Rockefeller Foundation and MAZON: A Jewish response to hunger.¹⁴ The VA is actively piloting produce prescription and MTM programs, demonstrating institutional validation of this

approach.¹⁵

Limye' Foundation's existing "Plates" MTM program is perfectly aligned with this VA priority.⁶ Rather than proposing a novel concept, the Foundation can position its "Plates for Patriots" program as a turnkey implementation partner for the VA's own strategic goals. Marketing materials and partnership proposals should utilize the VA's language of "whole health" and focus on improving outcomes for diet-related chronic conditions, signaling to the VA that Limye' is a knowledgeable partner ready to help them achieve their objectives.

Specific Service Market: Highly Rural Counties

To extend services beyond the Tuscaloosa-Birmingham corridor, the VA's Highly Rural Transportation Grants (HRTG) program is the key funding mechanism. This program provides grants of up to \$50,000 per eligible county to fund transportation for veterans in areas with a population density of fewer than seven people per square mile.⁴ This is an ideal vehicle for reaching veterans in surrounding counties like Greene, Hale, and Pickens.

However, federal regulations restrict HRTG eligibility to VA-recognized Veteran Service Organizations (VSOs) and State Veterans Service Agencies.⁵ As a 501(c)(3) non-profit, Limye' Foundation cannot apply directly for these funds. The only viable strategy is to forge a formal partnership with an established VSO, such as the Alabama state chapters of the American Legion, Disabled American Veterans (DAV), or Veterans of Foreign Wars (VFW).²¹ In this model, the VSO would serve as the prime applicant and grantee, with Limye' Foundation acting as the operational subcontractor providing the vehicles, drivers, and dispatch services—a structure permitted by the grant program. This necessitates a strategic outreach effort to secure a VSO partner, which becomes a critical dependency for the rural expansion component of this plan.

Table 2.H.1: Analysis of Veteran Service Opportunities					
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Service Area	Target Population	Primary Payer/Funder	Contracting Vehicle	Key Requirements	Strategic Approach for Limye'
Urban NEMT (Tuscaloosa/B'ham)	Veterans needing rides to VA or CCN appointments	U.S. Dept. of Veterans Affairs	VA Community Care Network (CCN) via Optum	Optum credentialing , NEMTAC-level driver/vehicle compliance, HSRM proficiency	Become an in-network Optum provider to receive direct referrals and bill for services. Target Tuscaloosa VA's suspended DAV service as an entry point.
Rural NEMT (Underserved Counties)	Veterans in counties like Greene, Hale, Pickens	U.S. Dept. of Veterans Affairs	Highly Rural Transportation Grant (HRTG)	Partnership with a recognized VSO, grant writing expertise, adherence to HRTG reporting	Form a subcontracting partnership with a VSO (e.g., American Legion) to be the operational arm for an HRTG application.
Medically Tailored Meals	Veterans with chronic conditions (e.g., diabetes, heart disease)	U.S. Dept. of Veterans Affairs, Grants	VA CCN via Optum, Foundation/Corporate Grants	RD-approved menus, HACCP kitchen, Optum credentialing , outcome tracking	Position "Plates for Patriots" as an implementation partner for the VA's "Food is Medicine" initiative.

					Seek referrals from VA dietitians and social workers.
Supportive Housing	Veterans at risk of or experiencing homelessness	HUD (VASH Vouchers), Grants, Private Donors	Master Lease Agreement, Grant Agreements	Partnership with property developer, wraparound case management, knowledge of housing subsidies	Partner with Duckworth-Morris to master lease a block of units, creating a stable client base for meal and transport services.

Section 3: Comprehensive Service Delivery Model

The original service programs—Healthcare Navigation, Comprehensive Nutrition Services, Multi-Modal Transportation, Youth and Family Services, and Housing and Community Support—remain as detailed in the initial blueprint.⁶ The following program is added as a new, integrated pillar of the Foundation's work.

Program 6: Limye' Veterans Initiative - "Operation Home Base"

Program Philosophy

Operation Home Base is an integrated care model designed to stabilize the lives of

veterans by addressing three core pillars of well-being: Housing, Health (Nutrition), and Access to Care (Transportation). The program's synergy is its key innovation. The housing component provides a stable client base and living environment. The nutrition component improves health outcomes and reduces medical crises. The transportation component ensures veterans remain connected to the VA healthcare system and the broader community. This closed-loop ecosystem creates efficiencies and amplifies the impact of each individual service.

Target Population

The program will serve honorably discharged veterans and their families in the West Alabama region who are experiencing or are at risk of homelessness, are facing food insecurity, or have chronic health conditions that require consistent, reliable access to VA medical care.

Component 1: Supportive Housing Partnership ("20/59 Ventures Housing Initiative")

- **Concept:** This initiative moves beyond mere housing assistance to secure a dedicated block of safe, affordable, and high-quality rental units specifically for veteran clients. The name "20/59 Ventures" is retained to honor the founder's vision and evoke the image of the I-20/59 corridor that connects the key service hubs of Tuscaloosa and Birmingham, symbolizing a pathway to stability.²²
- **Operational Partner:** To execute this, a partnership with a proven local real estate developer and property manager is essential. **Duckworth-Morris Real Estate** is identified as the primary target partner. Their extensive portfolio of over 4,000 managed units in Tuscaloosa and explicit experience with Section 8 housing demonstrates their capacity and familiarity with federal compliance protocols necessary for serving low-income populations.¹ Other potential partners in the Tuscaloosa area include Allied Realty and Attollo Homes.²⁴
- **Partnership Model:** Limye' Foundation will pursue a Master Lease Agreement with the selected partner for a set number of apartment units. The Foundation will then sublet these units to veteran clients enrolled in the Operation Home Base program. Limye' case managers will provide the intensive, wraparound support

services, while the real estate partner will be responsible for standard property maintenance, ensuring a high-quality living environment.

- **Client Flow and Sustainability:** Veterans housed through this initiative become a direct, reliable, and geographically concentrated customer base for the "Plates for Patriots" meal program and the "Vets-in-Motion" transportation service. This creates an efficient, closed-loop service model, reducing logistical costs and ensuring high utilization rates for the subsidiary programs.
- **Affordability:** The financial model will leverage existing federal housing subsidies, particularly the HUD-Veterans Affairs Supportive Housing (HUD-VASH) voucher program. Grant funding from sources like the Redstone Federal Credit Union Community Impact Grant Program ⁶ and community foundations will be sought to bridge any gaps and ensure rents remain deeply affordable for veteran clients.

Component 2: Veteran-Focused Nutrition ("Plates for Patriots")

- **Service Model:** This program is a specialized track of the Foundation's existing "Plates" Medically Tailored Meals (MTM) initiative.⁶ It will feature chef-prepared meals designed by a Registered Dietitian (RD) to meet the specific nutritional needs associated with chronic conditions common among veterans, such as diabetes, hypertension, and heart disease.¹⁴
- **VA Partnership Pathway:** A multi-pronged approach will be used to secure VA-funded clients:
 1. **Direct Referral:** Establish formal referral pathways with VA social workers, dietitians, and primary care providers at the Tuscaloosa and Birmingham VA Medical Centers.⁸
 2. **Community Care Network (CCN):** Aggressively pursue a contract with Optum to become a credentialed MTM provider. This will allow for direct billing for eligible veterans who are referred for MTM services through the CCN.
 3. **Pilot Program:** Propose a formal pilot project to the Tuscaloosa VA, leveraging the national "Food is Medicine" initiative.¹⁵ This pilot would focus on veterans in the "20/59 Ventures" housing units, allowing for the demonstration of a holistic care model with tightly controlled variables and measurable outcomes, such as reduced hospital readmissions or improved blood glucose (A1c) levels.
- **Client Base:** The initial and primary client base will be the residents of the "20/59 Ventures" housing units. This concentration dramatically increases logistical

efficiency for meal delivery, creating a dense and predictable delivery route.

Component 3: Veteran Transportation Services ("Vets-in-Motion")

- **Service Tiers:** The transportation program will operate on three distinct but integrated tiers:
 1. **Direct-to-VA NEMT:** This is the core billable service. It involves providing individual and small-group transport for veterans from their homes (including the 20/59 units) to medical appointments at the Tuscaloosa VA, Birmingham VA, and other VA-authorized community care provider offices. This service will be billed directly to the VA through the CCN via Optum.
 2. **Rural County Access (HRTG):** This tier focuses on expansion into underserved areas. By partnering with a VSO (e.g., American Legion Department of Alabama), Limye' will co-apply for HRTG funds to serve veterans in designated highly rural counties like Greene, Hale, and Pickens.⁴ Limye' will serve as the operational arm, providing the vehicles, certified drivers, and dispatching services under a subcontract with the VSO grantee.
 3. **Scheduled Inter-City Routes (Long-Term Vision):** The ultimate goal is to establish a fixed-route, scheduled shuttle service operating multiple times per week between Tuscaloosa and the Birmingham VA Health Care System.²⁷ This service would address a major regional transportation barrier for veterans needing specialized care available only in Birmingham. This can be marketed directly to the VTS coordinators at both the Tuscaloosa and Birmingham facilities as a cost-effective solution for their patient populations.⁷
- **Becoming a VA NEMT Provider:** This is the most technically demanding part of the initiative. The pathway involves:
 - **Contracting:** Initiating contact with Optum, the Region 3 TPA, to begin the network provider application and contracting process.³
 - **Credentialing:** Successfully completing Optum's credentialing process, which will rigorously assess the Foundation's licensing, insurance coverage, and operational readiness.³⁰
 - **Compliance:** Strict and continuous adherence to all VA and Optum requirements for NEMT providers, which are detailed in the Compliance section and the program SOP.

Section 4: Organizational Structure and Staffing

The Foundation's core organizational structure remains as outlined in the initial plan.⁶ To effectively manage the "Operation Home Base" initiative, the following additions and modifications are required.

Organizational Chart (Revised)

A new **Director of Veteran Services** will be added, reporting directly to the Chief Operating Officer. This director will have ultimate oversight for the entire Operation Home Base initiative and its integrated components. Reporting to this new director will be a **Veteran Services Case Manager / Intake Coordinator** and a **Lead NEMT Driver / Vets-in-Motion Coordinator**, creating a dedicated team for this population.

Key New and Modified Roles

- **Director of Veteran Services:** This senior position requires a candidate with significant experience in veteran benefits systems (VA, DoD), program management, and partnership development. This individual will be the primary liaison with the VA, Optum, and VSO partners.
- **Veteran Services Case Manager:** This frontline role requires a professional trained in trauma-informed care and possessing deep knowledge of VA resources, including the process for enrolling in VA healthcare via VA Form 10-10EZ.³² They will be the primary point of contact for veterans in the housing program, developing service plans and connecting them to resources.
- **Transportation Manager (Expanded Role):** This existing role from the original plan⁶ will have its responsibilities expanded to include ensuring full compliance with VA NEMT regulations, managing driver certifications (NEMTAC, CPR/First Aid), and overseeing the detailed reporting required for HRTG grants.
- **NEMT Drivers:** These are not standard drivers. They will require specialized training beyond a commercial license, including NEMTAC certification, Passenger Assistance Safety and Sensitivity (PASS), defensive driving, and specific training

on the needs of veterans with physical and psychological trauma.⁶

- **Grant Writer (Expanded Role):** This existing role from the original plan ⁶ must now possess or develop expertise in federal grant applications, specifically navigating Grants.gov for opportunities like the HRTG program and other VA-specific funding announcements.¹⁹

Table 4.1: Veteran Initiative Staffing & Training Plan				
Position Title	Key Responsibilities	Required Qualifications	Specialized Training Needed	Associated Cost (Training)
Director of Veteran Services	Oversee "Operation Home Base," manage VA/VSO partnerships, program reporting	5+ years in veteran services, program management, MSW or MPA preferred	VA Benefits Systems, Grant Management	\$2,000
Veteran Case Manager	Intake, assessment, ISP development, resource connection, client advocacy	BSW or related field, 2+ years case management experience	Trauma-Informed Care, Motivational Interviewing, VA Community Care Procedures	\$1,500
Vets-in-Motion Coordinator	Supervise NEMT drivers, manage dispatch, ensure vehicle compliance	3+ years in transportation logistics, dispatch experience	Fleet Management Software, NEMTAC Dispatch Certification, VA Reporting	\$1,000
NEMT Driver	Safe transport of veterans, passenger assistance, trip logging	Valid AL Driver's License, clean driving record, pass background check	NEMTAC Certification, CPR/First Aid, Defensive Driving, PASS Training	\$500 per driver

Grant Writer (Expanded)	Identify and write federal grant proposals, manage reporting	3+ years grant writing, proven federal grant success	Grants.gov System, HRTG Program Regulations, Federal Grant Compliance	\$1,000
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Section 8: Compliance and Regulatory Requirements

This section maintains all federal, state, and program-specific requirements outlined in the original business plan.⁶ The following subsection is added to address the complex regulatory environment of providing services to veterans through the VA.

D. VA Community Care and Transportation Compliance

Adherence to VA regulations is non-negotiable and will be a primary focus of the Director of Veteran Services and the Transportation Manager. Failure in this area could result in loss of contract and reputational damage.

- **VA CCN Provider Agreement:** The foundation must sign and meticulously adhere to a network provider agreement with Optum for CCN Region 3. This legally binding document will govern all aspects of the relationship, including referrals, authorizations, claims submission procedures, and payment schedules.³
- **HealthShare Referral Manager (HSRM):** Use of the VA's secure, web-based HSRM system is mandatory for receiving referrals and authorizations from the VA.³⁴ All relevant staff, including the Veteran Services Director, Case Manager, and transportation dispatch personnel, must be registered with ID.me accounts and fully trained on the HSRM platform to ensure seamless coordination of care.³⁴
- **NEMT Provider Requirements:** The "Vets-in-Motion" program must meet the VA's stringent requirements for transportation providers, which are more rigorous than standard NEMT.³⁰
 - **Insurance:** The Foundation must secure tiered commercial auto liability insurance that meets or exceeds VA minimums. Based on standards in other states, this is projected to be at least \$350,000 for vehicles carrying 1-6

passengers and \$1,500,000 for vans carrying 7-15 passengers.³⁰ This has a direct and significant impact on the operating budget.

- **Vehicle Standards:** All vehicles used for veteran transport must be fully ADA-compliant, properly registered, and subject to daily pre-trip safety inspections and a documented preventative maintenance schedule.²⁰
- **Driver Qualifications:** All drivers must possess a valid state driver's license and pass comprehensive screening, including driving record checks, criminal background checks, health screenings (DOT physicals), and a random drug testing program. They must hold and maintain current certifications in CPR, First Aid, and Defensive Driving.³⁰
- **Highly Rural Transportation Grant (HRTG) Compliance:** If successful in securing an HRTG subcontract, the Foundation will be subject to strict federal grant reporting requirements. This includes submitting detailed quarterly and final reports to the VSO partner and the VA, documenting trips completed, unique veterans served, total distance covered, and a full accounting of how grant funds were expended.²⁰
- **HIPAA and Veteran Privacy:** All staff must receive enhanced training on HIPAA and the specific privacy rules protecting veteran data, which are often more stringent than standard healthcare privacy laws.⁶

Table 8.D.1: VA NEMT Provider Compliance Checklist			
Compliance Area	Requirement	Verification Method	Responsible Staff
Business Registration	Active registration in SAM.gov with a valid UEI.	Annual SAM.gov profile review and update.	CFO / Director of Veteran Services
Insurance	Commercial Auto Liability meeting VA tiers (e.g., \$350k/\$1.5M).	Certificate of Insurance on file, reviewed quarterly.	Transportation Manager
Vehicle Standards	ADA-compliant; daily safety inspection logs; preventative maintenance records.	Vehicle files with all documentation; random audits.	Transportation Manager / Lead Driver

Driver Qualifications	Valid License, CPR/First Aid Cert, clean background/drug screen, DOT physical.	Driver personnel files with all credentials; annual review.	Transportation Manager / HR
Documentation	Accurate trip logs (patient, times, mileage); incident reports within 24 hours.	Digital records in fleet management software; monthly report review.	Transportation Manager / Dispatcher
VA Systems Access	Staff registered and active on HSRM and Optum Provider Portal.	Quarterly user access review.	Director of Veteran Services

Section 14: Standard Operating Procedures (SOP) for Limye' Veterans Initiative

14.1 Recruitment and Intake Strategy SOP

Outreach Channels

1. Primary Outreach (Direct VA Partnership):

- The Director of Veteran Services will establish formal Memoranda of Understanding (MOU) and referral protocols with the Tuscaloosa VA Medical Center ⁸ and the Birmingham VA Health Care System.²⁷
- Key points of contact for these referrals will be the Social Work departments ²⁶, Homeless Veteran Coordinator offices, and the Office of Community Care at each facility.¹³ Regular in-person meetings will be scheduled to maintain

relationships and ensure a steady flow of appropriate referrals.

2. **Secondary Outreach (VSO Partnership):**

- The Director will actively engage with local Veteran Service Organizations in the Tuscaloosa-Birmingham corridor, including posts for the American Legion, Veterans of Foreign Wars (VFW), and Disabled American Veterans (DAV).⁹
- The strategy includes attending monthly meetings to present the "Operation Home Base" program and distribute materials, establishing Limye' as a trusted community resource.

3. **Tertiary Outreach (Community Engagement):**

- Build relationships with other veteran-centric community entities, such as the Birmingham Vet Center in Hoover ⁹ and the University of Alabama's Campus Veterans Association ³⁹, to reach veterans who may not be actively engaged with the VA medical system.

Intake Process

1. **Initial Contact & Screening:** A veteran is referred by a partner or self-refers via phone or website. The Intake Coordinator conducts an initial screening to confirm basic eligibility: veteran status (any discharge other than dishonorable), residence within the service area, and the nature of the expressed need (housing, food, transport).
2. **Benefits Verification:** The Veteran Services Case Manager meets with the veteran. The first step is to verify enrollment in VA healthcare. If the veteran is not enrolled, the Case Manager will assist them in completing and submitting VA Form 10-10EZ.³² The Case Manager will then help determine the veteran's eligibility for community care based on VA criteria.¹⁰
3. **Comprehensive Assessment:** The Case Manager administers a comprehensive intake assessment using a standardized tool (modeled after the PRAPARE tool mentioned in the original plan ⁶) to identify and score needs across multiple domains: housing stability, food security, transportation access, physical and mental health status, and employment.
4. **Individualized Service Planning (ISP):** Based on the assessment, the Case Manager and veteran collaboratively develop an ISP. This plan explicitly outlines which Limye' services the veteran will engage with (e.g., placement in "20/59 Ventures" housing, enrollment in "Plates for Patriots," scheduling for "Vets-in-Motion") and sets clear goals.
5. **Program Enrollment:** The veteran signs all necessary internal program consent

forms and releases of information required for care coordination with the VA and other partners.

14.2 Supportive Housing Program ("20/59 Ventures") SOP

1. **Client Eligibility:** The veteran must be enrolled in the Operation Home Base program with a signed ISP and agree to participate in ongoing case management. Priority for placement is given to veterans who are currently homeless, at imminent risk of homelessness, or fleeing domestic violence.
2. **Placement Process:**
 - The Case Manager identifies an available, appropriate unit within the block of apartments master-leased from the property partner (e.g., Duckworth-Morris).
 - The Case Manager assists the veteran with the sublease application and all required paperwork for housing subsidies, such as a HUD-VASH voucher application packet.
 - Upon approval, the Case Manager coordinates the move-in date, key exchange, and initial inspection with the property management company.
3. **Ongoing Case Management:**
 - The Case Manager will conduct in-person check-ins with the veteran weekly for the first month, bi-weekly for months two and three, and monthly thereafter, adjusting frequency based on need.
 - The Case Manager's role is to ensure the veteran is connected to the full suite of Limye' services and external resources for employment, mental health counseling (e.g., Birmingham Vet Center ³⁸), and other needs.
 - The Case Manager proactively monitors for any issues related to lease compliance and serves as a mediator between the veteran and property management to prevent eviction and ensure housing stability.
4. **Rent and Subsidy Management:** The finance department will establish a clear process for collecting the veteran's portion of the monthly rent (if any) and for receiving and processing housing subsidy payments directly from the Public Housing Authority or other relevant agencies.

14.3 NEMT Services ("Vets-in-Motion") SOP

1. Booking a Ride (CCN Referrals):

- The process begins when the VA approves a veteran for a community care appointment and generates a referral/authorization, which is transmitted to Limye' via the HealthShare Referral Manager (HSRM) portal.³⁴
- The veteran, their VA scheduler, or their Limye' Case Manager calls the "Vets-in-Motion" dispatch line (or uses an online portal) to schedule the ride.
- Required information for booking: Veteran's full name, VA authorization number, appointment date, time, and location, pickup address, and a description of any mobility needs (e.g., wheelchair, walker).
- The dispatcher verifies the authorization number in HSRM, confirms the ride in the fleet management software, provides a pickup window to the caller, and assigns the trip to a driver.

2. Pre-Trip Procedure:

- At the start of each shift, the driver conducts and signs a daily vehicle safety inspection using a standardized checklist (tires, lights, lift, safety equipment).
- The driver reviews their electronic manifest for the day, noting any specific passenger assistance requirements or notes for each trip.

3. Trip Execution:

- The driver arrives at the pickup location approximately 15 minutes prior to the scheduled departure time.
- The driver provides courteous, professional, curb-to-curb assistance, including operating the wheelchair lift and ensuring the passenger is safely secured, in full compliance with ADA standards.
- The driver transports the veteran to their appointment safely and efficiently.
- Before leaving the veteran at the drop-off point, the driver confirms the procedure for the return trip (e.g., "Call dispatch when the appointment is finished, and we will send the next available driver").

4. Post-Trip and Billing:

- Immediately following each leg of the trip, the driver accurately logs trip data (pickup/drop-off times, odometer readings) into the fleet management system via a mobile tablet.
- Daily, an administrative staff member reviews the completed trip logs. They reconcile the data with the VA authorization numbers and submit electronic claims to Optum through their provider portal for payment.²⁸
- The administrative staff member is responsible for tracking claim status, appealing any denials, and posting payments.

5. HRTG and Scheduled Route Procedures:

- **HRTG Trips:** These trips are booked through the same dispatch system but are flagged as "grant-funded." They do not have individual VA authorization numbers. Trip data (mileage, veteran served) is compiled separately for the quarterly HRTG report.³⁶
- **Scheduled Shuttle:** This service operates on a fixed route and schedule. Veterans can reserve a seat in advance, but walk-ons are accepted if space is available. No authorization is required, as this is envisioned as a broader community service, potentially funded by grants or a nominal fee.

14.4 Medically Tailored Meal Program ("Plates for Patriots") SOP

1. Enrollment:

- A veteran is identified as a candidate for MTMs during the comprehensive intake assessment or via a direct referral from a VA provider (e.g., dietitian, primary care physician).
- A valid referral from a VA provider must include a qualifying medical diagnosis (e.g., Type 2 Diabetes, Congestive Heart Failure) and a dietary prescription (e.g., "Carbohydrate-controlled, low-sodium diet").¹⁴
- The Limye' Foundation's Registered Dietitian (RD) reviews the referral, contacts the veteran to confirm preferences and allergies, and formally assigns them to the appropriate meal plan (e.g., Diabetic-Friendly, Heart-Healthy, Renal).

2. Production and Delivery:

- The Kitchen Manager receives the weekly list of enrolled veterans and their assigned meal plans.
- The kitchen staff prepares all meals in the commercial kitchen, strictly adhering to the RD-approved recipes and portion controls for each specific diet.
- Meals are individually packaged and labeled with the veteran's name, the meal type, and heating instructions, in compliance with all health department regulations.
- The Transportation Director uses logistics software to create an optimized delivery route, with a primary focus on the "20/59 Ventures" housing cluster for maximum efficiency.
- A dedicated meal delivery driver conducts deliveries. At each stop, the driver performs a brief visual welfare check and can report any concerns back to the veteran's Case Manager.

3. Billing and Reporting:

- For veterans referred through the CCN, the administrative staff submits claims to Optum based on the service authorization, billing for the number of meals delivered.
- For veterans whose meals are funded by grants or donations, service delivery data is meticulously tracked for funder reporting.
- On a quarterly basis, the Limye' RD provides a summary report to the referring VA provider detailing the veteran's adherence and any reported outcomes, closing the loop on the care coordination process.

Conclusion

The integration of the "Operation Home Base" initiative into the Limye' Foundation's comprehensive service model represents a pivotal evolution for the organization. This blueprint provides a detailed, actionable, and research-backed strategy to address the complex, interconnected needs of Alabama's veterans. By moving beyond siloed services and creating a synergistic ecosystem of housing, nutrition, and transportation, the Foundation is positioned to deliver transformative, measurable outcomes.

The plan acknowledges and provides clear pathways to navigate the significant complexities of partnering with the Department of Veterans Affairs, specifically the critical necessity of engaging with the CCN administrator, Optum, and forging strategic alliances with VSOs to access specialized funding like the HRTG. The detailed Standard Operating Procedures provide the granular, on-the-ground guidance necessary for successful implementation, risk mitigation, and compliance.

Successful execution of this plan will not only provide life-changing support to hundreds of veterans and their families but will also establish Limye' Foundation as a leader in innovative social service delivery. It creates a robust, financially sustainable model that leverages diverse revenue streams—from federal healthcare reimbursements to grant funding—to fuel its mission. This initiative will illuminate a new pathway to possibility, not just for the veterans we serve, but for the future of the Foundation itself.

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